2. Wound CTP (Skin Substitute Grafting) Note

📄 **Wound Care Documentation Templates**

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1. Wound Care Assessment Follow Up Visit Note

Patient Information

Field	Entry
Patient Name	
MRN	
DOB	
Date of Service	
Provider	
Location	

Reason for Visit / Chief Complaint

Field	Details
Reason for visit	e.g., "Application of cellular/tissue-based product (CTP) for chronic non-healing diabetic foot ulcer."

History of Present Illness (HPI)

Parameter	Details
Onset of wound	
Duration	
Standard care attempted	≥4 weeks (list all: debridement, dressings, compression, glucose control)

Parameter	Details
Compliance	Good / Fair / Poor
Current wound status	

Physical Exam

Review of Systems

Symptom	Present (Y/N)
Fever/Chills	
Malaise	
Pain at wound site	
Drainage/Odor	
Other systemic complaints	

Notes

Physical Exam

Parameter	Findings
General	Well-appearing / Ill-appearing
Vitals	BP: ___ / HR: ___ / Temp: ___ / SpO_2: ___
Extremities	Edema, pulses, sensation, mobility

Vascular Assessment Findings: (ABI, pulses, Doppler studies if indicated)

Complete At Risk Foot Exam

Class Findings:

Class B: (1) decreased hair growth, nail thickening, and discoloration

Class C: (2) edema and cold feet

The patient meets Class Findings Criteria by having at least 1 Class B Finding and 2 Class C Findings.

Gait: standard walker. limited household ambulator.

Right Shoe Size: 12.0

Width: Wide

Measuring Device: Brannock Device

Left Shoe Size: 12.0

Width: Wide

Measuring Device: Brannock Device

At Risk Foot Exam

Peripheral Pulses:

Right Dorsalis Pedis: +1 faint but detectable dorsalis pedis pulse

Right Posterior Tibial: +1 faint but detectable posterior tibial pulse

Right Capillary Refill: Normal CFT

Right Vascular Skin Trophic Changes: absent hair

growth, decreased temperature gradient, pigmentation changes, and cool

Right Edema: Non pitting

Right Venous Exam: telangiectasia

Sensation:

Right LE: Normal peripheral nerve sensation

Peripheral Pulses:

Left Dorsalis Pedis: +1 faint but detectable dorsalis pedis pulse

Left Posterior Tibial: +1 faint but detectable posterior tibial pulse

Left Capillary Refill: Normal CFT

Left Vascular Skin Trophic Changes: absent hair growth, decreased temperature gradient, pigmentation changes, and cool

Left Edema: Non pitting

Left Venous Exam: telangiectasia

Sensation:

Left LE: Normal peripheral nerve sensation

Skin:

Right Foot and Ankle: ulcer Wound bed granular less fat layer within the wound bed than previous. Does not probe deep. No fluctuance or purulence noted. No

other local signs of infection.

Toenails:

Right: Normal

Inspection:

Right Foot and Ankle: Charcot arthropathy

Appearance: well developed and nourished

Orientation: Alert and oriented to person, place, time.

Additional Exam Findings:

- Wound Width Pre-treatment in cm: 1.9

- Wound Depth Pre-treatment in cm: .3

- Wound Length Pre-treatment in cm: 2.9

Skin:

Left Foot and Ankle: there is normal texture, temperature, turgor and color of the skin

Toenails:

Left: Normal

Inspection:

Left Foot and Ankle: Charcot Arthropathy

Wound Examination (Required Per Wound)

Wound #	Location	Etiology	Stage	Length (cm)	Width (cm)	Depth (cm)	Surface Area (cm^2)	Volume (cm^3)

Wound Bed	Granulation %	Slough %	Eschar %	Exudate (type/amount)	Odor (Y/N)	Periwound Condition	Undermining/Tunneling	Pain on Palpation	Signs of Infection

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Assessment

Clinical Summary	Details
Wound type and severity	e.g., “Chronic Stage III venous ulcer, right medial ankle.”
Contributing factors	Diabetes, immobility, venous stasis
Risk factors if untreated	Infection, hospitalization, limb loss
Medical necessity justification	Document why continued care is required

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Procedure Performed (Separate for each Graft/ Debridement, etc)

Step	Details
Indication	Chronic non-healing wound unresponsive to ≥4 weeks of SOC

Step	Details
Consent	Obtained / Risks & benefits discussed
Preparation	Wound cleansed, sharp debridement performed
Hemostasis achieved	Y/N
CTP applied	Product name, size, lot #, expiration
Total Units of graft applied per wound	000 Units(total cm of graft)
Application details	Trimmed to wound bed, secured with [non-adherent dressing/bolster/NPWT]
Secondary dressing	Type
Tolerance	Patient tolerated well with [local anesthesia, etc.]
Estimated blood loss	Minimal
Complications	None
Any Graft wastage	None

Medical Necessity

Field	Details
Wound status	Chronic, non-healing, refractory to standard care
Meets LCD criteria?	Yes (SOC ≥4 weeks, infection controlled, adequate vascular supply)
Medical necessity	Prevents progression, hospitalization, or amputation

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Plan

Treatment	Details
Cleansing method	

Treatment	Details
Debridement type	Sharp / Enzymatic / Autolytic
Dressing applied	Type and frequency
Offloading/Compression	
Nutritional guidance	
Pain control	
Follow-up schedule	Weekly / Biweekly
Escalation plan	If inadequate progress → advanced therapy (NPWT, HBOT, CTP grafting)

Plan / Post-Procedure Care

Field	Details
Dressing instructions	Keep intact for [X days]
Offloading/compression	Continue as prescribed
Monitoring	Watch for infection, graft displacement
Comorbidity optimization	Glucose, vascular, nutrition
Follow-up	Weekly wound assessment, additional CTP if medically necessary

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Diagnosis:

Diagnosis	Code	Comments
Pressure ulcer of left buttock, stage 4	L89.324	None
-	-	-

Diagnosis	Code	Comments
-	-	-

Lab tests:

-	Lab name	Test Name
-	General	CBC

Follow up:

Type	Date	Notes
Home/office	9/25/27	None

Name- Referring Provider

Name- Primary care provider

Staff:

Name-Primary provider (Bill Under)

Other Photos

Left heel ulcer f/u pre	Moleculight post debridement	Moleculight pre
Post debridement left heelulcer	Serial numbers	Skin sub application_1
Skin sub application_2	[redacted] Snapshot post debridement	[redacted] Snapshot pre

SHORELINE medical administration

Medicare Compliance Checklists by Jurisdiction

Illinois (IL) – NGS MAC

Relevant LCDs

- **L38902 – Wound Care**

- **L35041 – Skin Substitute Grafts/Cellular and Tissue-Based Products (CTPs) for the Treatment of Diabetic Foot Ulcers and Venous Leg Ulcers**

Wound Care Assessment Checklist (IL)

☐ Wound etiology documented (DFU, VLU, PU, trauma, surgical, arterial).
☐ Wound measurements (length, width, depth, surface area, volume).
☐ Wound characteristics (granulation %, slough %, eschar %, exudate type/amount, odor, periwound).
☐ Documentation of **skilled intervention** (not just dressing change).
☐ Debridement (if performed): method + % tissue removed.
☐ Medical necessity stated (risk of progression, infection, limb loss).
☐ Pain score documented.
☐ Patient comorbidities (diabetes, vascular disease, CKD, etc.).
☐ Evidence that wound requires ongoing skilled care, not healing by conservative management.
☐ Follow-up plan documented (frequency + escalation if no progress).

📌 **IL References**:

- **LCD L38902 – Wound Care**
- **NCD 270.1 – Wound Care**

CTP / Skin Substitute Checklist (IL)

☐ SOC ≥4 weeks documented before first application.
☐ SOC included: debridement, moist wound dressings, infection control, off-loading (DFU) or compression (VLU), vascular assessment, comorbidity management.
☐ Weekly wound measurements during SOC.
☐ Wound measurements documented before each graft.
☐ No untreated infection at time of grafting.
☐ Wound bed prepared (free of necrosis/eschar).
☐ CTP product details: brand, size, lot #, expiration.
☐ Limit: ≤8 applications per 12–16 week episode; >4 requires provider attestation + proof of progression.
☐ Medical necessity: documented risk of hospitalization, amputation, or failure to heal without CTP.
☐ Follow-up plan: wound reassessment weekly, additional CTP if medically necessary.

📌 **IL References**:

- **LCD L35041 – Skin Substitute Grafts/CTPs**
- **NCD 270.3 – Skin Substitute Grafts**

Georgia (GA) – Palmetto GBA MAC

Relevant LCDs

- **L38765 – Wound Care**
- **L36377 – Skin Substitute Grafts/CTPs for the Treatment of Diabetic Foot Ulcers and Venous Leg Ulcers**

Wound Care Assessment Checklist (GA)

☐ Wound type documented (DFU, VLU, PU, trauma, surgical, arterial).
☐ Wound dimensions (L/W/D, SA, volume).
☐ Wound characteristics (granulation %, slough %, exudate, odor, periwound).
☐ Skilled intervention documented (beyond routine dressing).
☐ Debridement documentation (type + % tissue removed).
☐ Comorbidities assessed and managed.
☐ Pain documented.
☐ Evidence of medical necessity: wound not healing, risk of infection/hospitalization/limb loss.
☐ Plan of care: frequency + escalation path.

📌 **GA References**:

- **LCD L38765 – Wound Care**
- **NCD 270.1 – Wound Care**

CTP / Skin Substitute Checklist (GA)

☐ SOC ≥4 weeks documented prior to graft.
☐ SOC included: debridement, dressings, infection control, off-loading (DFU) or compression (VLU), vascular evaluation, comorbidity management.
☐ Wound dimensions documented weekly during SOC and before each graft.
☐ Infection excluded or controlled before grafting.
☐ Wound bed prepared (granulation tissue present, necrosis removed).
☐ Product information: brand, size, lot #, expiration documented.
☐ Maximum applications: ≤8 per 12–16 week episode; >4 requires documentation of wound progression.
☐ Medical necessity: wound refractory to SOC, risk of deterioration, hospitalization, or amputation.
☐ Follow-up documented: wound reassessment and graft monitoring.

📌 **GA References**:

- **LCD L36377 – Skin Substitute Grafts/CTPs**
- **NCD 270.3 – Skin Substitute Grafts**

Key LCDs / LCAs & NCDs

1. Wound Care LCD: L38902

- Covers wound and ulcer care when wounds are refractory (not healing properly) or complicated by metabolic/physiologic factors. (Centers for Medicare & Medicaid Services)
- Requires active wound care management including debridement, infection control, evaluation of complicating factors (vascular supply, nutrition, pressure off-loading). (Centers for Medicare & Medicaid Services)

2. Skin Substitute Grafts / Cellular & Tissue-Based Products LCD: L35041

- Applies for DFUs (Diabetic Foot Ulcers) & VLUs (Venous Leg Ulcers) that have failed standard of care (SOC) for at least **4 weeks**. (Centers for Medicare & Medicaid Services)
- SOC must include vascular assessment, infection control, off-loading / compression (for VLUs), management of comorbidities (e.g. diabetes, etc.). (Centers for Medicare & Medicaid Services)
- Templates for documenting ulcer measurements before, during (~weekly), and at time of grafting, plus continuing SOC during and after grafting. (Centers for Medicare & Medicaid Services)
- Maximum number of applications per "episode of skin replacement therapy" (12–16 weeks) is typically **8**. More than 4 require provider attestation and evidence of progression. (Centers for Medicare & Medicaid Services)

3. No National Coverage Determination (NCD) overrides these for skin substitute grafts in most cases — the policies are local (state/MAC-based). (UHC Provider)

Medicare Compliance Checklist — Illinois & Georgia

☐ Wound etiology clearly documented (DFU, VLU, pressure ulcer, trauma, etc.).
☐ Wound has been present for at least **4 weeks** with standard of care (SOC) therapy documented and patient's compliance noted.
☐ Standard of care documented and includes:

- Debridement (sharp / selective / non-selective) as needed.
- Infection control / no untreated infection.
- Compression therapy for venous ulcers, or off-loading for diabetic foot ulcers.
- Vascular assessment (e.g. ABI, pulses, doppler studies if indicated).
- Management of comorbidities (e.g. diabetes, nutrition, smoking).

☐ Wound measurements documented:
- Length, width, depth (cm)
- Surface area (cm^2)
- Volume if applicable

☐ Weekly ulcer measurements during SOC period (before grafting).
☐ Measurements at time of CTP application, and before each subsequent CTP application.

☐ Product details for skin substitute / CTP documented: brand name, size, lot number, expiration date.

☐ Number of CTP applications in the episode of care documented; not exceeding **8** in the 12–16 week span unless justified with attestation and evidence of healing progression.

☐ Application method documented correctly (dressing, securing graft, wound bed preparation).

☐ Evidence of improvement (granulation, epithelialization, reduction in wound dimensions/exudate) during SOC and post-CTP application.

☐ Follow-up plan well documented: schedule, how to monitor graft, how to continue or escalate therapy.

☐ Provider qualifications: the provider performing or ordering grafts / advanced treatments is appropriately licensed, trained, and within MAC jurisdiction.

☐ All documentation legible, with patient identifiers, date(s) of service, and signed (or compliant digital signature) by the provider responsible.